



Fundamentals of Medicine I

FUNDAMENTALS OF MEDICINE 1 MSM 2022--2022

Overall Sequence Director	Dr. M. Elks	Medical Education x1881	melks@msm.edu
Human Values 1	Dr. M. Elks	Medical Education x1881	melks@msm.edu
Epidemiology/Biostatistics	M. Elks		
Human Behavior	Dr. Nicole Cotton & Dr. Marietta Collins	Psychiatry & FM	
Wellness Modules	Dr. M. Elks and others		
Intro to patient diagnosis	Dr. M. Elks and others		
Clinical Preceptorship	M. Davis CHPM,		mdavis@msm.edu
Learning Communities	Dr. A. Kennedy-Mitchell and others		akennedy@msm.edu
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IPE (interprofessional education)	Dr. M. McNeal and others		mmcneal@msm.edu
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Fundamentals of Medicine 1 is a year-long interdisciplinary course that blends together Human Values 1, Human Behavior, Epidemiology/Biostatistics, Clinical Preceptorship, introductory patient diagnosis, Wellness modules, IPE (interprofessional education) and our learning communities. These component courses/ streams form the foundation for core clinical skills. Each of the eight components has a course director and explicit requirements for completion. There are also interdisciplinary elements include explicit instruction in basic clinical skills and simulated (online iHuman) cases that are linked to the integrated curriculum (linked to Gross Anatomy stream). There will be a mix of lecture, discussion, skills sessions, clinical sessions, self-directed modules and small group experiences. Grady day experience will be independently scheduled for an in-person Saturday session, as circumstances permit.

The **learning communities** include curricular elements in this course and in Community Health. The Learning Communities span all experiences of the first year and will extend in curricular and extra-curricular experiences across training. Small group experiences in Fundamentals and in some other courses will align with Learning Community groups.

The year-long course has an irregular schedule. There are extended component/courses in the first two curricular blocks that build the fundamentals in epidemiology, biostatistics, ethics, the health care systems, and basic clinical skills in preparation for the rest of the year. Didactics are predominantly online (live Zoom and prerecorded asynchronous modules). From September to March, the students participate in monthly clinical experiences in the community that are the "laboratory" for understanding the relationships and context of these fundamental concepts and skills (Clinical Preceptorship). These include required HEAL clinic sessions (HEAL is the MSM student run clinic—the scheduling of sessions is through that clinic.) Introductory sessions on history taking and approaches to patient diagnosis are started in the fall. Key physical exam skills and linked cases are aligned with gross anatomy sessions and neurobiology sessions in the spring. There are five iHuman required cases (asynchronous online) that allow practice of history taking skills for cases aligned with the Gross Anatomy and integrated curriculum organ system. The behavioral sciences sequence (Human Behavior) extends across the fall to the spring and provides context for the field experience in interviewing geriatric patients and for the practice of communication skills/behavioral change with standardized patients. Wellness modules are online learning units required of all students with associated quizzes that must be passed. There are elective workshops and a small personal health improvement project. Grady Day and HEAL clinic sessions are required for this course and will be on a separate individual schedule and do not appear on the syllabus schedule.

As adherence to credentialing process is a key competency, students are required to complete modules for clinical site credentialing in ACEMAPP. There are specific due dates for uploading some information and completing certain training. **Failure to complete these modules on schedule will result in a letter grade reduction. Failure to complete them in time for schedule clinical activities will result in an unexcused absence that will also reduce the grade.** Students who do not complete the modules will fail FOM1.

The role of Learning Community experiences linked to this course include opportunities to build skills with reflection, career planning, professionalism, active listening, conflict management, stress management, giving and receiving feedback, and teamwork skills.

This course is focused on building skills and attitudes as well as knowledge and most of the learning experiences are experiential. **As such, attendance is required at experiential sessions (all clinical preceptorship, field experience, physical exam experiences, learning communities, and standardized patient experiences, and Grady day). Attendance will be taken by roll call, sign in, zoom log, or similar method.** All absences are subject to remediation (repeat or alternative experience). **Unexcused absences of required sessions will result in a lowering of final grade in that area, and may result in failure of the course.** (grade will be lowered by 1 grade level for each instance of an unexcused absence). Excused absences must be in accordance with school policies and must be documented in writing (copy must be placed on file with the Sequence Director and the Course Director). Students must contact the Course Director in writing prior to an expected absence and receive permission for the expected excused absence. For an emergency absence, students are expected to contact the course director in writing as soon as it is feasible to do so (and follow school policies on excused absences through the dean for Student Affairs). It is the student's responsibility to communicate with the responsible Course sequence director to make arrangements for making up missed experiences. **Unexcused absences over a stated threshold will result in course failure.** **(Please note that some required attendance is “ZOOM” virtual participation, and some will be in person on campus or at the assigned site.)**

Interprofessional sessions (IPE) will be small group across the year. The Schedule will be updated.

Most didactic sessions will be by live lecture or zoom (or similar online) session. Selected elements will be pre-recorded and can be studied by the learner at any time. Skills sessions with live patient actors will be on a specific schedule by student that will be provided proximate to the events. Live skills sessions and some learning communities must be in person (excused absence will need make-up live session).

Under the circumstances of the pandemic and changing CDC recommendations, timing and nature of required in-person experiences is subject to modification.

Course competencies (objectives)--Upon completing the course, the student will be able to:

1. Define profession, and define actions important to developing and sustaining a professional attitude in training and as one grows as a physician. Identify some of the key challenges, stresses, and changes faced by physicians in training and describe some approaches to dealing with these issues. (MD learning objective 1 d)
2. Identify and define principles guiding ethical clinical decision-making, including beneficence, nonmaleficence, autonomy, and justice, and their relationship to underlying philosophical theories

- and application in health care and research (MD Learning Objective 1 d)
3. For common clinical ethical dilemmas, identify the competing principles and outline an approach to decision-making, taking into consideration ethical, legal, personal, religious, ethnic, and communication issues. (MD Learning Objective 1 d)
 4. Describe the principles and approaches for maintenance of confidentiality and for informed consent in clinical settings and research, complete HIPAA training, and display appropriate confidentiality
 5. Define conflict of interest and outline approaches to supporting ethical actions in situations that pose a conflict of interest. (MD Learning Objective 1 d)
 6. Demonstrate professionalism in all interactions with faculty, peers, staff and patients, including integrity, truthfulness, dependability, adherence to relevant standards, and related behaviors. (MD Learning Objective 1 d)
 7. Demonstrate basic skills in communication with peers and patients and an understanding of the complexities of the physician-patient relationship. (MD Learning Objective 2 D)
 8. Demonstrate active listening and conflict management skills, and interpersonal and teamwork skills. (MD Learning Objective 2 d)
 9. Demonstrate good communication skills in taking a health history and in health promotion. (MD Learning Objective 1 d)
 10. Briefly describe health-care funding in the United States and the challenges of supporting health care for underserved populations addressing health disparities and health equity (MD Learning Objectives 1 d, 3)
 11. Demonstrate basic skills in history taking and documentation in patient note. (MD Learning Objective 2a)
 12. Demonstrate basic clinical skills in physical examination and in documentation in the patient note. (MD Learning Objective 2 a & 2d)
 13. Define "primary care" and discuss the variety of reasons for patients' visits with primary care practitioners. Identify the roles of the health care team members that support the primary care physician. (MD Learning Objective 1 d)
 14. Describe the common ethnic and cultural views of health care in the populations we serve and describe ways the physician can show respect for these views and deliver patient-centered care. (MD Learning Objective 1 d)
 15. Describe and discuss normal human development throughout the life cycle and the role and interaction of families and the impact of these on health issues. (MD Learning Objective 1 a & 1 c)
 16. Identify and describe psychological, sociological, economic, environmental, religious, and demographic variables that impact human health behavior and beliefs. Demonstrate how to approach a case from a biopsychosocial perspective. (MD Learning Objective 1 d)
 17. Identify some key issues in interaction with geriatric patients and in geriatric care and demonstrate skills in communication with this population. (MD Learning Objective 2 e)
 18. Apply basic epidemiological concepts and method, including prevalence, incidence, sensitivity, specificity, relative rate reduction, and so forth in exercises identifying, and assessing public health problems. (MD Learning Objective 1c)

19. Identify and describe the correct use and application of basic biostatistical concepts and methods, including the normal distribution, t-tests, chi square tests, correlation, and so forth in the evaluation of public health, clinical and research problems. (MD Learning Objective 1 c)
20. Demonstrate the access and analysis of relevant medical information from library and computer-based resources for addressing a medical question. (MD Learning Objective 1 c)
21. Describe basic practices in wellness (including diet, exercise, sleep, connectedness, and safety) and identify at least 1 way a physician can support a patient in adopting these practices. (MD Learning Objective 1d)
22. Demonstrate knowledge of the roles of other health professions in the care of the patient and in advancing health equity and demonstrate interprofessional teamwork skills. (1 d, 2 f)
23. Demonstrate the skills of Self-Directed Learning (SDI) including self-assessment of needs, identification, analysis, synthesis, and appraisal of resources, sharing with faculty and peers, self-reflection on process, and response to feedback from faculty (1.d, 2 f))
24. Demonstrate compliance with routine ambulatory infection-control measures including up to date immunization, adherence to credentialing processes and timelines, proper use of hand-washing, facial masks, gloves, social distancing and other issues. (MD Learning Objective 1 b)

Required Texts: B. Fadem, BRS Behavioral Science 7th or 8th ed. **REQUIRED FOR FINAL EXAM FOR FOM2!!!!!!!!!!!!!!!!!!!!!!!!!!!!**

Bickley, L et al, Bates Guide to History Taking and Physical Diagnosis, 12th or 13th ed (also used in year 2—is available online from library)

Recommended: A. Glaser, High-Yield Biostatistics, 3rd ed or other edition

Grading

Grading will be based on exams (multiple choice in the integrated exams) performance of specified reports or papers, and attendance at required sessions. Attendance and participation in the FoM1 Learning Community (LC) sessions is required. Explicit separate syllabi will be provided for each integral course. These will specify the exact requirements for each. Each course will contribute to the full final course grade and **each course must be passed to complete the full Course Sequence. (ie—a student with 100% average in all components but 69.4% (or less) in one course will receive an F in the course (failing)).**

A comprehensive essay take-home final will be given at the end of the year. Attendance, as noted above is required for many of the sessions. All absences for required experiences must be excused. **More than one unexcused absence will result in a failing course grade.**

To pass the HV1 component/course, students need to score at least 70% of the exam points and attend all sessions (make-up excused absences.)

To pass the Epidemiology/biostatistics component/course, students need to score at least 70% of the exam points and attend all sessions (make-up excused absences.)

To pass the Human Behavior component/course, students need to score at least 70% of the exam points and attend all sessions (make-up excused absences.)

To pass the CP component/course, one must participate in all sessions (or make-up for excused absences), and complete all required paperwork.

To pass the Wellness component/course, student must achieve passing score on the quizzes, participate in at least 1 wellness workshop, and complete at least 1 wellness self-improvement project.

To pass the ihuman component, students must satisfactorily complete the 5 simulated cases

To pass the PD (physical Diagnosis) component, students need to participate in the skills sessions, demonstrate appropriate skills, and score at least 70% on exam points.

To pass the SDI (self-directed learning) component, students must satisfactorily document (satisfactory or better on all components) 5 instances of SDI

To pass the Learning communities component one must participate in all LC sessions and must display satisfactory skills (communication, teamwork, professionalism, self-directed learning).

To pass the IPE component, students must participate in assigned sessions and complete linked assignments

To pass the ACEMAPP credentialing, students must adhere to the timelines specified through the Canvas web site. Points are not ascribed to this section, but failure will result in course failure.

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Statement for Disability Services

Morehouse School of Medicine seeks to provide students with disabilities equal opportunities and equal access to academic programs, services, and activities. If you have a disability for which you wish to request accommodations, please contact the Office of Disability Services at the beginning of each semester (or as soon as practicable). The Office of Disability Services is located in NCPC, Room 408. You may contact the office at (404) 756-5200 or via e-mail at ODS@msm.edu.

Remediation

In most instances, remediation of a failure of the course will require learning activities and documentation of skills (as in note, standardized patient encounters, case simulation encounter or similar skills assessment) as aligned with all the key competencies that the learner has not shown evidence of mastering. There is in-course-enrichment for students with difficulty with epidemiology-

biostatistics concepts. Remediation of HB or HV sessions will be by self-study and exam. Remediation of the other components will require individually aligned experiences, skills sessions, and satisfactory product submission as noted for the component/course.

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(blue- —required participation-classroom with zoom)

Peach—prerecorded/asynchronous modules with PPT)

(Black-in person LC, CP or physical diagnosis)

(green—in person small group)

EX—questions from this session will be on BP or OS exams

THIS SCHEDULE WILL CHANGE-info emailed and posted on CANVAS SITE

Tues 7/5 11:00-12:30 (HV 1) Introduction/ becoming an doctor at MSM (Elks) (EX)

1:30-3:00 (LC) Intro; Learning Communities—Introductions

Wed 7/6 1:30-3:00 (HV1) Health care system (Elks) (EX)

Thur 7/7 3:30-5:00 (LC) –1:30-3P different days by LC

Or 7/8 based on LC

Tues 7/12 asynchronous (WE) Wellness and /study skills: modules (Elks)

Wed 7/13 asynchronous (EB) Epidemiology online (Tables/Elks/online) (EX)

Tues 7/19 EXAM (combined exam with BP)

WED 7/20 3:30-500 (LC) (ON WED 7/20 Thur 7/21)

Thur 7/21 asynchronous (EB) Epi/Bios online (EX)

Thur 7/21 9-10:30 (IPD) Introduction to patient assessment –wellness (Elks) EX

Tues 7/27 Wellness project plan due; wellness quizzes due

Wed 7/28 1:30 -3 (HV) Mastering medicine—exam review strategy (Mork/Redding)

Mon 8/1- *asynchronous* (EB) Hypothesis testing-Biostatistics online (EX

Thur 8/4 1:30-5:00 (HB) Introduction to HB (EX)

Mon 8/8 EXAM

Tues 8/9 *asynchronous* (HB) Biological theory (Cotton) (EX

THur 8/11 1:30-3, 3:30-5 (LC)

Tues 8/16 12:30-3p (LC) year 1 & 2 mingle & lunch

Wed 8/17 ACEMAPP COMPLETION DUE DATE

Wed 8/17 *asynchronous* (HB) Social Theory (Vinson) EX

Thur 8/18 *asynchronous* (HB) Psychological Theory (Collins) (EX)

Tues 8/23 EXAM

Thur 8/25/Fri 26 1:30-5 (IPD) skills in clinical skills center (EX) (by groups

Thur 8/25/Fri 25 1:30-3 (by groups) Myers Briggs

Fri 8/26 9-10:30 (HV1) Physician as professional/Ethics-White coat ceremony (Elks)

Fri 8/26 11-12:30 (LC)

Mon 8/29 9-10:30 (HV1) beginning and end of life issues (Elks)

Tues 8/30 9-10:30 (CP) orientation to CP

8/31 on (PD) with Gross anatomy—schedule variable EX

Mon 9/12 9-10:30 (HV1) Justice & the Health care; Impacts of racism(Elks) (EX)

Mon 9/12 1:30-3 (LC) Ethics

Mon 9/12		FoM1 wellness plan report due (upload to canvas)
Tues 9/13 or	1:30-5	CP or Skills
Thur 9/15		
Fri 9/16	10-12	WHITE COAT CEREMONY
Thur 9/22	9-5	(CP) Clinical Preceptorship or (LC) Cultural experience
		Detailed schedule to be provided
Mon 9/26	<i>asynchronous</i>	(HB) The Biopsychosocial model (Collins) EX
Mon 9/27	PM	(or thur 9/30 or Mon 10/4) (gross &)) PD –back & arm exam
Mon 9/27	<i>asynchronous</i>	(PD) start I human case 1, do intro modules
Wed 10/5	11-12:30	(HB) Substance Use and Behavior Change (Cotton) (Zoom)
Thur 10/7	9-12:30 or	(CP) Clinical Preceptorship
	2-5	
Wed 10/12		EXAM
Thur 10/13	2-5	(CP)
Tues 10/18	2-5	(CP)
Wed 10/19	11-12:30	(LC) career panel
Fri 10/21	asynchronous	(EB)hypothesis testing review for exam (EX)
Mon 10/24 or 27	12:30-1:30	(IPE)
Tues 10/25	9-12, 2-5	(CP) or LC
Fri 10/28	asynchronous	(HB) Human Development
Thur 10/29	PM	or mon 11/2 PM or thur 11/5 P (Gross & PD) heart/lung exam

Fri 10/29	11-12:30	(HB) Substance abuse and Behavioral Change (Cotton) (EX)
Tues 11/2or 11/3 9-12:30		(LC) OR (HB) smoking cessation (NCPC e-lab)) or Pulm function lab
Mon 11/7		EXAM
Tues 11/9	9-5	(CP) Clinical Preceptorship or LC
Thur 11/11	PM	or Fri 11/12—Gross and Abd exam
Tues 11/15	1;30-5	(CP) Clinical Preceptorship or OS1 problem solving
Thur 11/17	1:30-5	(CP) Clinical Preceptorship or OS1 problem solving
Fri 12/8		EXAM
TBA		With Gross—pelvic exam; /Male GU (simulators)
TBA		with Gross—LE exam
Tues1/17	9-10:30	(HB) Human Sexual development (Bayer)EX
Mon 1/23	<i>asynchronous</i>	((HB) Human Development-part 2 (Cotton) EX
Thur 1/26		EXAM
Tues 1/31	9-5	(LC) or (CP) or (HB) Development Skills Lab
Tues 2/7	9-5	(LC) or (CP) or (HB) Development Skills Lab
Tues 3/7	2-5:30	(CP) Clinical Preceptorship or IPE
Thur 3/9	2-5:30	(CP) Clinical Preceptorship or IPE
Wed 3.28	9-10:30	(LC)
Tues 4/4	1:30-5	(LC) or CP

Thur 4/6	1:30-5	(LC) or (CP)
Tues 5/23	12N	Take home exams due
Tues 5/16	2-5:30	(CP) Clinical Preceptorship (make-up)